

Target validation paths — hgsoc-tuboovarian-stage3c-r0-hrd-pen ding-idyq

The diagnostic and biomarker workup that hardens the targetable-feature call

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Target validation paths

Every biomarker in this case has resulted. One workup is still worth ordering: a validated genomic-instability score, which is the only test that decides whether olaparib plus bevacizumab (PAOLA-1, NCT02477644) is an option. If a Myriad MyChoice CDx GIS returns at least 42, that combination becomes available; below 42 the tumor reads HRD-negative and the combination is foreclosed, leaving all-comer PARP, anti-angiogenic, endocrine, and on-study options unaffected. The remaining rows below record assays that have already resulted, kept here so the workup picture is complete.

HRD / genomic instability

The Altera tumor NGS calls genomic instability qualitatively, reporting chromosomal imbalances and frequent LOH, but it gives no validated quantitative score, and a qualitative call does not meet the companion-diagnostic definition of HRD-positive. This matters because the PAOLA-1 olaparib-plus-bevacizumab indication defines HRD-positive as either a deleterious BRCA mutation or a Myriad MyChoice CDx GIS of at least 42. The tumor is BRCA-wild-type, so eligibility rests entirely on the GIS. This is the one essential, decision-gating workup left: it runs on archival FFPE, with the surgical resection block preferred for tumor content, turnaround 2 to 3 weeks. The MyChoice CDx is the only assay whose threshold maps directly to the olaparib-plus-bevacizumab label. FoundationOne CDx and Caris report LOH-based HRD surrogates, so confirm the score maps to the at-least-42 threshold before relying on either for eligibility. Germline 74-gene panel and tumor BRCA1/2 sequencing have both resulted as wild-type, which forecloses BRCA-gated olaparib monotherapy (SOLO1) and routes the PARP decision onto this score; no further BRCA testing is outstanding.

ER

ER is positive in roughly 80 percent of nuclei at weak-to-moderate intensity with PR negative, which supports a low-toxicity aromatase-inhibitor option such as letrozole. PR-negativity predicts a more modest endocrine response, so the semiquantitative ER/PR readout sets expectations rather than gating a drug. The values are already reported; no additional staining is needed unless a later specimen is obtained.

PIK3CA / MAP2K4

The Altera panel reported focal PIK3CA amplification and focal MAP2K4 deletion, both investigational-only in HGSC with no approved matched therapy. They matter only as potential early-phase trial-matching tags on the PI3K/AKT/mTOR axis or MAP2K4-listed studies. No confirmatory or orthogonal assay is warranted; the findings are recorded for trial-screening completeness.

Signatera ctDNA (MRD)

Tumor-informed ctDNA has cleared to Not Detected after cytoreduction and chemoimmunotherapy, marking a favorable molecular-residual-disease state that informs how intensively to pursue maintenance. Serial monitoring can flag biochemical relapse ahead of CA-125 or imaging. It does not select a target or gate a drug, so this is surveillance rather than a one-time gating assay; continuity favors keeping draws on the platform that designed the patient-specific panel.

Where to order these assays

Assay	Provider	Decision gated	Contact
Validated genomic-instability score (Myriad MyChoice CDx GIS, or FDA-approved equivalent companion diagnostic)	Myriad Genetics (preferred) (MyChoice CDx)	Olaparib plus bevacizumab maintenance (PAOLA-1) eligibility in BRCA-wild-type HGSC, which is gated on a validated GIS of at least 42.	test info · 320 Wakara Way, Salt Lake City, UT 84108 · 1-800-469-7423
Validated genomic-instability score (Myriad MyChoice CDx GIS, or FDA-approved equivalent companion diagnostic)	Foundation Medicine (FoundationOne CDx (LOH-based HRD))	Olaparib plus bevacizumab maintenance (PAOLA-1) eligibility in BRCA-wild-type HGSC, which is gated on a validated GIS of at least 42.	test info · 150 Second Street, Cambridge, MA 02141 · 1-888-988-3639
Validated genomic-instability score (Myriad MyChoice CDx GIS, or FDA-approved equivalent companion diagnostic)	Caris Life Sciences (Caris HRD / genomic LOH)	Olaparib plus bevacizumab maintenance (PAOLA-1) eligibility in BRCA-wild-type HGSC, which is gated on a validated GIS of at least 42.	test info · 4610 South 44th Place, Phoenix, AZ 85040 · 1-888-979-8669
Serial tumor-informed ctDNA MRD (Signatera, already cleared to Not Detected)	Natera (preferred) (Signatera)	Maintenance intensity and relapse surveillance; does not gate a specific drug.	test info · 13011 McCallen Pass, Building A, Austin, TX 78753 · 1-650-980-9190

Biomarker plan

Recommended assay	Rationale	Suggested assay provider	Tissue requirements
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Validated genomic-instability score (Myriad MyChoice CDx GIS, or FDA-approved equivalent companion diagnostic)	The Altera tumor NGS calls genomic instability qualitatively (chromosomal imbalances and frequent LOH) but reports no validated quantitative score, and a qualitative call does not satisfy the companion-diagnostic definition of HRD-positive. The PAOLA-1 maintenance indication requires HRD-positive status defined as a deleterious BRCA mutation or a Myriad MyChoice CDx GIS of at least 42; this tumor is BRCA wild-type, so eligibility rests entirely on the GIS. Without a validated score the patient defaults to all-comer niraparib maintenance (PRIMA) and the higher-benefit option in HRD-positive disease cannot be offered.	Myriad Genetics (MyChoice CDx) · test info · 320 Wakara Way, Salt Lake City, UT 84108 · 1-800-469-7423	archival FFPE acceptable (surgical resection block preferred for tumor content)
Germline 74-gene panel plus tumor BRCA1/2 sequencing (already performed)	Both germline (74-gene panel) and somatic (Altera tumor/normal exome) BRCA1/2 testing have resulted as wild-type, which is the resolution that previously gated the case. This forecloses the BRCA-mutation-gated indications (olaparib monotherapy maintenance, SOLO1) and shifts the PARP question onto the genomic-instability score. No further BRCA testing is needed; the row records the resolved state so downstream agents do not re-flag it as pending.	Resolved on germline blood and tumor block already tested; no additional provider order required	none additional; germline blood and tumor block already tested

ER and PR semiquantitative IHC (Allred or percentage plus intensity, already reported)	ER is positive in roughly 80 percent of nuclei at weak-to-moderate intensity with PR negative, which supports a low-toxicity aromatase-inhibitor option (for example letrozole) as maintenance or later-line therapy that fits the patient's age and oral-preference. PR-negativity predicts a more modest endocrine response, so the quantitative ER/PR readout sets expectations rather than gating a drug. The values are already reported; no additional staining is required unless a later specimen is obtained.	Reported on the diagnostic IHC panel; no additional provider order required	archival FFPE acceptable
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Comprehensive tumor NGS reporting PIK3CA amplification and MAP2K4 deletion (already performed, Altera)	The Altera panel reported focal PIK3CA amplification and focal MAP2K4 deletion, both investigational-only in HGSC with no approved matched therapy. They are relevant only as potential early-phase trial-matching tags (PI3K/AKT/mTOR-axis or MAP2K4-listed studies). No confirmatory or orthogonal assay is warranted; the findings are recorded for trial-screening completeness.	Reported on the Altera tumor NGS panel; no additional provider order required	archival FFPE acceptable
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Serial tumor-informed ctDNA MRD (Signatera, already cleared to Not Detected)	Signatera tumor-informed ctDNA has cleared to Not Detected after cytoreduction and chemoimmunotherapy, which marks a favorable state and informs how intensively to pursue maintenance. Serial monitoring can detect biochemical relapse ahead of CA-125 or imaging and help time maintenance decisions. It does not select a target or gate any specific drug, so priority is medium and surveillance, not a one-time gating assay.	Natera (Signatera) · test info · 13011 McCallen Pass, Building A, Austin, TX 78753 · 1-650-980-9190	serial whole-blood draws; tumor block already used for assay design
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Decision support, not medical advice. Confirm assay availability and current standards with the treating team and the local pathology service.
